

1. Information about the indicator	
Goal	Ensure healthy lives and promote well-being for all at all ages
Task	By 2030, reduce the global maternal mortality ratio to less than 70 per 100,000 live births
The indicator	Maternal mortality ratio
2. Information about the organization	
Organization	Ministry of Healthcare of the Republic of Kazakhstan
3. Definitions and concepts	
Definition	The maternal mortality ratio (MMR) is defined as the number of maternal deaths during a given time period per 100,000 live births during the same period. This indicator reflects the risk of maternal death relative to the number of live births and essentially captures the risk of death resulting from a single pregnancy or live birth.
Basic concepts:	The annual number of deaths of women from any cause related to pregnancy occurring during pregnancy and childbirth, or within 42 days after termination of pregnancy, regardless of the duration and site of the pregnancy, expressed as a relative value per 100,000 live births in the same time period. Pregnancy-related death: The death of a woman while pregnant or within 42 days of termination of pregnancy, regardless of the cause of death. Late maternal death: The death of a woman from direct or indirect obstetric causes more than 42 days, but less than one year, after termination of pregnancy.
Justification and interpretation	All maternal mortality indicators obtained in the 2015 assessment stage include a point estimate and an 80% uncertainty interval (UI). For those indicators presented in the text or tables in the form of point estimates, the uncertainty interval can be filled in using additional materials from the Internet http://www.who.int/reproductivehealth/publications/monitoring/maternal-mortality-2015/en/). When analyzing data, both point estimates and the 80% uncertainty interval should be taken into account.
4. Data sources and collection methods	
Data sources	Administrative data.
Data collection methods	Registration of civil status acts.
Unit of measurement	per 100,000 live births
Periodicity	Monthly
5. Calculation method and other methodological considerations	
Calculation method:	The maternal mortality ratio is calculated by dividing the registered maternal deaths by the total number of registered live births for the same period and multiplying by 100,000. Measurement requires information on pregnancy status, timing of death (during pregnancy, childbirth, or within 42 days of termination of pregnancy), and its cause. All maternal mortality indicators obtained in the 2015 assessment stage include a point estimate and an 80% uncertainty interval (UI). For those indicators presented in text or tables in the form of point estimates, the uncertainty interval can be supplemented using additional materials from the Internet (http://www.who.int/reproductivehealth/publications/monitoring/maternal-mortality-2015/en/).

	mortality-2015/en/). When analyzing data, both point estimates and the 80% uncertainty interval should be taken into account. For example: The estimated global MMR in 2015 was 216 (uncertainty interval 207 to 249). Breakdown: Point estimate – 216, with an 80% uncertainty interval ranging from 207 to 249. There is a 50% chance that the true global MMR in 2015 is above 216 or below 216. There is an 80% probability that the true global MMR in 2015 lies between 207 and 249. There is an additional 10% probability that the true global MMR in 2015 is above 249 or below 207. Other precise evaluations include: A 90% probability that the true global MMR in 2015 is at least 207. A 90% probability that the true global MMR in 2015 is 249 or less.
Comments and restrictions:	
6. Data availability and disaggregation	
Data availability and gaps:	The data is available on the website of the Bureau of National Statistics of the Agency for Strategic Planning and Reforms of the Republic of Kazakhstan.
The level of disaggregation:	The data is disaggregated by geographical location.
7. Comparability with international data/standards	The maternal mortality ratio is defined as the number of maternal deaths divided by the number of live births. However, to account for potential incompleteness in death registration across various data sources, the Maternal Mortality Estimation Inter-Agency Group (MMEIG) first calculates the proportion of deaths due to maternal causes from the source data (referred to as the "maternal proportion" or MP) and then applies this function to the WHO estimate of overall mortality among women of reproductive age to derive an estimate of the number of maternal deaths. In other words, the following function is first calculated from the country's data sources: $MP = \text{Number of maternal deaths among women aged 15–49} / \text{Total deaths among women aged 15–49}$ and then the MP is used to calculate the MMR as follows: $MMR = MP * (\text{Total deaths among women aged 15–49} / \text{Number of live births})$ where the estimate for total deaths among women aged 15–49 in the second equation is derived from WHO life tables, and the number of live births is taken from the <i>World Population Prospects 2015</i>. Several reasons can be cited for why MMEIG estimates may differ from national statistical data: Civil status registration systems and vital statistics are not

	always complete (i.e., they do not always capture 100% of all deaths), and completeness can change over time. The MMEIG estimation approach attempts to correct for this using the aforementioned method, which involves first calculating the MP. 2. MMEIG frequently applies adjustment factors to the MP—calculated based on source data—to account for measurement issues (e.g., how a country defines maternal deaths, misclassification, or underreporting). 3. MMEIG uses standardized live birth figures from the UN Population Division, published in the <i>World Population Prospects 2015</i> report, in the denominator of the MMR equation. To better inform the <i>World Population Prospects</i>, countries should discuss discrepancies directly with the UN Population Division. <i>Contact address:</i> population@un.org; this email address is regularly monitored, and messages are forwarded to the respective analysts for each country or administrative unit. 4. Statistical rarity: Maternal mortality is a relatively rare event, which can lead to distorted data trends over time. Since the goal of MMEIG estimates is to track long-term progress in reducing maternal mortality, the estimation process involves some smoothing to generate a curve that better reflects changes in the underlying risk.
8. References and documentation	https://stat.gov.kz/ru/sustainable-development-goals/goal/3/